Patient Name	**: Mrs ANAR DEVI**		
DOB/Age/Gender	: 60 Y/Female	Bill Date	: Sep 15, 2023, 02:59 PM
Patient ID / UHID	: 5476580/OF5476580	Sample Collected	: Sep 15, 2023, 02:59 PM
Referred By	: Dr.AMIT KUMAR PANDEY	Sample Received	: Sep 15, 2023, 04:36 PM
Sample Type	: Whole blood EDTA	Report Date	: Sep 15, 2023, 05:54 PM
Barcode No	: HX188383	Report Status	: Final Report

MC-5280

HEMATOLOGY REPORT

BharatFit Package- 2

Complete Blood Count (CBC)

Test Description	Value(s)	Unit(s)	Reference Range
RBC PARAMETERS			
Hemoglobin Method : colorimetric	12.5	g/dL	12.0 - 15.0
RBC Count Method : Electrical impedance	4.3	10^6/µl	3.8 - 4.8
PCV Method : Calculated	38.9	%	36 - 46
MCV Method : Calculated	89.9	fl	83 - 101
MCH Method : Calculated	28.9	pg	27 - 32
MCHC Method : Calculated	32.2	g/dL	31.5 - 34.5
RDW (CV) Method : Calculated	13.6	%	11.6 - 14.0
RDW-SD Method : Calculated	42.4	fl	35.1 - 43.9
WBC PARAMETERS			
TLC Method : Electrical impedance and microscopy	6.2	10^3/µl	4 - 10
DIFFERENTIAL LEUCOCYTE COUNT			
Neutrophils	67	%	40-80
Lymphocytes	24	%	20-40
Monocytes	3	%	2-10

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Test Description	Value(s)	Unit(s)	Reference Range
Method : Calculated			
PCT	**0.1**	%	0.17 - 0.32
Method : Calculated			
PDW	**32.3**	fL	8.3 - 25.0
Method : Calculated			
P-LCR	**61.7**	%	18 - 50
Method : Calculated			
P-LCC	66	%	44 - 140
Method : Calculated			
Mentzer Index	20.91		

Interpretation:
CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

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MC-5280

Test Description	Value(s)	Unit(s)	Reference Range

HEMATOLOGY REPORT

BharatFit Package- 2

HbA1C (Glycosylated Haemoglobin)

GLYCOSYLATED HEMOGLOBIN (HbA1c) Method : HPLC	**6.2**	%	< 5.7
ESTIMATED AVERAGE GLUCOSE	131.24	mg/dL	Refer Table Below

Interpretation:

Interpretation For HbA1c% As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments :

HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

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Referred By	: Dr.AMIT KUMAR PANDEY	Sample Received	: Sep 15, 2023, 04:36 PM
Sample Type	: FLUORIDE F	Report Date	: Sep 15, 2023, 05:31 PM
Barcode No	: BY670503	Report Status	: Final Report

MC-5280

Test Description	Value(s)	Unit(s)	Reference Range

BIOCHEMISTRY REPORT

BharatFit Package- 2

Glucose Fasting (BSF)

GLUCOSE FASTING Method : Hexokinase	**188.3**	mg/dL	Normal <100 Prediabetes 100–125 Diabetes >=126

Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	<100
Impaired fasting glucose	100 - 125
Diabetes	=>126

Reference : American Diabetes Association

Comment :
Blood glucose determinations in commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy insulinoma, or various liver diseases.

Note
1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL or a random / 2 hour plasma glucose value of > or = 200 mg/dL with symptoms of diabetes mellitus.
2.Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.

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Referred By	: Dr.AMIT KUMAR PANDEY	Sample Received	: Sep 15, 2023, 04:36 PM
Sample Type	: Serum	Report Date	: Sep 15, 2023, 05:39 PM
Barcode No	: BY670504	Report Status	: Final Report

MC-5280

BIOCHEMISTRY REPORT

BharatFit Package- 2

Liver Function Test (LFT)

Test Description	Value(s)	Unit(s)	Reference Range
BILIRUBIN TOTAL Method : Diazo	0.4	mg/dL	0 - 1.2
BILIRUBIN DIRECT Method : Diazo Jondrof	0.2	mg/dL	0 - 0.20
BILIRUBIN INDIRECT Method : Calculation (T Bil - D Bil)	0.2	mg/dL	0.1 - 1.0
SGOT/AST Method : IFCC without P5P	20.1	U/L	up to 32
SGPT/ALT Method : IFCC without P5P	22.2	U/L	up to 33
SGOT/SGPT Ratio Method : Calculated	0.91	-	-
ALKALINE PHOSPHATASE Method : IFCC	97.8	U/L	35 - 104
TOTAL PROTEIN Method : Biuret	6.78	g/dL	6.4 - 8.3
ALBUMIN Method : BCG Colorimetric	4.59	g/dL	3.5 - 5.2
GLOBULIN Method : Calculation (T.P - Albumin)	**2.19**	g/dL	2.3 - 3.5
ALBUMIN : GLOBULIN RATIO Method : Calculation (Albumin/Globulin)	2.1	-	1.0 - 2.1
GAMMA GLUTAMYL TRANSFERASE (GGT) Method : IFCC Colorimetric	23	U/L	5 - 36

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Test Description	Value(s)	Unit(s)	Reference Range

2.liver disorder
3.malnutrition
4.agammaglobulinemia High Protein levels 'Hyperproteinemia: May be seen in dehydration due to inadequate water intake or to excessive water loss (eg, severe vomiting, diarrhea, Addison's disease and diabetic acidosis) or as a result of increased production of proteins Low albumin levels may be

caused by:
1.A poor diet (malnutrition).
2.Kidney disease.
3.Liver disease. High albumin levels may be caused by: Severe dehydration.

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MC-5280

BIOCHEMISTRY REPORT

BharatFit Package- 2

Kidney Function Test (KFT)

Test Description	Value(s)	Unit(s)	Reference Range
BLOOD UREA Method : Urease with UV	43.6	mg/dL	16.6 - 48.5
CREATININE Method : Jaffes	0.76	mg/dL	0.50 - 0.90
BUN Method : Calculated	20.37	mg/dL	8 - 23
BUN/CREATININE RATIO Method : Calculated	**26.8**		5.5 - 19.2
UREA / CREATININE RATIO	57.37		
URIC ACID Method : Uricase	5.6	mg/dL	2.4 - 5.7
CALCIUM Serum Method : BAPTA	9.06	mg/dL	8.8 - 10.2
PHOSPHORUS Method : Molybdate UV	**2.38**	mg/dL	2.5 - 4.5
SODIUM Method : ISE-Indirect	136.89	mmol/L	136 - 145
POTASSIUM Method : ISE-Indirect	**3.04**	mmol/L	3.5 - 5.1
CHLORIDE Method : ISE-Indirect	**97.8**	mmol/L	98 - 107

Please correlate clinically.

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BIOCHEMISTRY REPORT

BharatFit Package- 2

Lipid Profile

Test Description	Value(s)	Unit(s)	Reference Range
TOTAL CHOLESTEROL Method : CHOD-PAP	178.3	mg/dL	Desirable : <200 Borderline : 200-239 High : >240
TRIGLYCERIDES Method : Enzymatic colorimetric	140.8	mg/dL	Normal : <150 Borderline : 150-199 High : 200-499 Very high : >500
HDL CHOLESTEROL Method : CHOD-POD	64.9	mg/dL	Low : < 40 High : >=60
NON HDL CHOLESTEROL Method : Calculated	113.4	mg/dL	<130
LDL CHOLESTEROL Method : Calculated	85.24	mg/dL	Optimal <100 Near optimal/above optimal 100-129 Borderline high 130-159 High 160-189 Very high >190
V.L.D.L CHOLESTEROL Method : Calculated	28.16	mg/dL	< 30
CHOL/HDL Ratio Method : Calculated	**2.75**	Ratio	3.5 - 5.0
HDL/ LDL RATIO Method : Calculated	0.76	-	Desirable : 0.5 - 3.0 Borderline : 3.1 - 6.0

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Test Description	Value(s)	Unit(s)	Reference Range

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >/= 50 mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age >or=45 years in males and > or = 55 years in females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	**Treatment Goals**		**Consider Drug Therapy**	
	LDL-C (mg/dl)	**Non-HDL (mg/dl)**	**LDL-C (mg/dl)**	**Non-HDL (mg/dl)**
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130

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Sample Type	: Serum	Report Date	: Sep 15, 2023, 05:39 PM
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MC-5280

Test Description	Value(s)	Unit(s)	Reference Range

BIOCHEMISTRY REPORT

BharatFit Package- 2

Thyroid Profile Total

TRIIODOTHYRONINE (T3) Method : ECLIA	121	ng/dL	80 - 200
TOTAL THYROXINE (T4) Method : ECLIA	9.35	µg/dL	5.1 - 14.1
THYROID STIMULATING HORMONE (Ultrasensitive) Method : ECLIA	3	mIU/L	0.27 - 4.20

Interpretation:

Pregnancy	Reference ranges TSH
1 st Trimester	0.1 - 2.5
2 ed Trimester	0.2 - 3.0
3 rd Trimester	0.3 - 3.0

Primary malfunction of the thyroid gland may result in excessive (hyper) or below normal (hypo) release of T3 or T4. In addition as TSH directly affects thyroid function, malfunction of the pituitary or the hypo - thalamus influences the thyroid gland activity. Disease in any portion of the thyroid-pitutary-hypothala- mus system may influence the levels of T3 and T4 in the blood. In primary hypothyroidism, TSH levels are significantly elevated, while in secondary and tertiary hypothyroidism, TSH levels may be low. In addition, in the Euthyroid Sick Syndrome, multiple alterations in serum thyroid function test findings have been recognized in patients with a wide variety of non-thyroidal illnesses (NTI) without evidence of preexisting thyroid or hypothalami c-pitutary diseases. Thyroid Binding Globulin (TBG) concentrations remain relatively constant in healthy individuals. However, pregnancy, excess estrogen's, androgen's, antibiotic steroids and glucocorticoids are known to alter TBG levels and may cause false thyroid values for Total T3 and T4 tests.

TSH	T4	T3	INTERPRETATION
High	Normal	Normal	Mild (subclinical) hypothyroidism
		Low or	

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Test Description	Value(s)	Unit(s)	Reference Range

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Referred By	: Dr.AMIT KUMAR PANDEY	Sample Received	: Sep 15, 2023, 04:36 PM
Sample Type	: Spot Urine	Report Date	: Sep 15, 2023, 06:32 PM
Barcode No	: CI691092	Report Status	: Final Report

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Test Description	Value(s)	Unit(s)	Reference Range

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Patient Name	: **Mrs ANAR DEVI**		
DOB/Age/Gender	: 60 Y/Female	Bill Date	: Sep 15, 2023, 02:59 PM
Patient ID / UHID	: 5476580/OF5476580	Sample Collected	: Sep 15, 2023, 02:59 PM
Referred By	: Dr.AMIT KUMAR PANDEY	Sample Received	: Sep 15, 2023, 04:36 PM
Sample Type	: Spot Urine	Report Date	: Sep 15, 2023, 06:32 PM
Barcode No	: CI691092	Report Status	: Final Report

MC-5280

CLINICAL PATHOLOGY REPORT

BharatFit Package- 2

Urine Routine and Microscopic Examination

Test Description	Value(s)	Unit(s)	Reference Range
PHYSICAL EXAMINATON			
Volume	20	mL	-
Colour	Pale yellow	-	Pale yellow
Transparency	Clear	-	Clear
Deposit	Absent	-	Absent
CHEMICAL EXAMINATION			
Reaction (pH) Method : Double Indicator	5.5	-	4.5 - 8.0
Specific Gravity Method : Ion Exchange	1.010	-	1.010 - 1.030
Urine Glucose (sugar) Method : Oxidase / Peroxidase	Negative	-	Negative
Urine Protein (Albumin) Method : Acid / Base Colour Excahnge	Negative	-	Negative
Urine Ketones (Acetone) Method : Legals Test	Negative	-	Negative
Blood Method : Peroxidase Hemoglobin	Negative	-	Negative
Leucocyte esterase Method : Enzymatic Reaction	Negative	-	Negative
Bilirubin Urine Method : Coupling Reaction	Negative	-	Negative
Nitrite	Negative		Negative

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Storage

Targeted Advertising

Personalization

Performance Cookies

Save Preferences

Accept All

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